Documentation Review Package

Pre-sign-off quality review of a Dragon Copilot draft clinical encounter note

Patient	Vance, Eleanor · DOB 04/12/1958 · MRN 904821
Encounter	ENC-2026-1014-884 · 10/14/2026 · Outpatient chronic disease follow-up
Clinician	Marcus Thorne, MD
Reviewer role	Clinical office coordinator (first-line documentation QA)
Sources reviewed	(1) Source conversation transcript, Document 1A; (2) Dragon Copilot draft clinical encounter note, unreviewed; (3) EMR post-transfer chart scenario, PracticeOne EMR v12.4
EMR status at review	Pending final clinician reconciliation and sign-off
Disposition	**ESCALATE** (Section 4)

16 corrections annotated in Section 1	**10** discrepancies logged, 3 of them high severity	**6** EMR integrity findings across 3 dimensions	**Escalate** disposition: not ready for routine sign-off

Review method

1. The source transcript was read in full before the draft was opened. Key moments were marked: the lisinopril change, the diabetes review, the travel counseling and the follow-up instructions.
2. The draft was annotated one section at a time (History, then Assessment, then Plan). Every correction quotes the transcript passage that supports it.
3. The discrepancy log was built with the transcript and the annotated draft open side by side.
4. The EMR chart was then evaluated separately for field appropriateness, traceability and downstream usability.
5. The disposition draws only on findings already documented in Sections 2 and 3.

Package contents

- **Section 1** Annotated Clinical Encounter Note
- **Section 2** Discrepancy Log
- **Section 3** EMR Record Integrity Evaluation
- **Section 4** Disposition Recommendation

Section 1 — Annotated Clinical Encounter Note

The Dragon Copilot draft is reproduced below with corrections shown as a fixed redline, so it reads the same in Word, PDF and print. Each correction carries a numbered tag that links to the annotation table under its section. That table names the documentation standard involved (**completeness**, **clinical encounter relevance** or **clinical specificity**), quotes the supporting transcript passage and cross-references the discrepancy log (E1–E10). The draft's closing "Note for Annotation Exercise" is instructional text and is not reproduced.

Key: ~~deleted text~~ · inserted text · A1 annotation reference

Note header

> **Document Type:** Unreviewed Ambient AI Draft Output
>
> **Encounter Date:** October 14, 2026
>
> **Patient:** Vance, Eleanor (DOB: 04/12/1958)
>
> **Provider:** Marcus Thorne, MD
>
> **Encounter Type:** Outpatient Chronic Disease Follow-Up

Header verified against the transcript and EMR: name, DOB, encounter date and provider match. No corrections.

History (Subjective)

> **Chief Complaint:** Follow-up for Type 2 Diabetes and Essential Hypertension, including recent lisinopril dose adjustment; new dizziness on standing. A1
>
> **History of Present Illness:** Patient is a 68-year-old female presenting for ~~routine~~ chronic disease follow-up of type 2 diabetes and essential hypertension, approximately 4 weeks after lisinopril was increased from 10 mg to 20 mg daily. A2 Reports ~~mild~~new morning dizziness upon standing quickly, resolving within a few seconds. A3 No GI upset reported on metformin ER. A4 Patient discussed an upcoming 2-week cruise to Alaska next month and ~~requested travel clearance and medication management advice for the trip~~asked whether any special medication precautions are needed for travel. A5

ID	Correction and reason	Standard	Transcript evidence	Log
A1	Added the recent lisinopril dose adjustment and the patient's new complaint to the chief complaint. Both were the stated reasons for this visit.	**Clinical encounter relevance**	*Reason for Visit: "Recent medication adjustment." Dr. Thorne: "especially since we increased your Lisinopril about four weeks ago."*	E6
A2	Removed "routine." Added the lisinopril increase (10 → 20 mg, about 4 weeks ago) that prompted the visit. The prior 10 mg dose is implied by "back down… to 10 mg"; verify against medication history at order entry.	**Clinical encounter relevance**	*"we increased your Lisinopril about four weeks ago"; "step your Lisinopril back down from 20 mg daily to 10 mg daily."*	E6

ID	Correction and reason	Standard	Transcript evidence	Log
A3	Removed "mild," which the patient never said. Added onset, trigger and duration. The patient did not time the onset to the dose increase, so that link is kept in the Assessment as the clinician's reasoning rather than added to the History.	**Clinical specificity**	*Mrs. Vance: "dizziness when I stand up quickly in the morning. It passes after a few seconds, but it's new."*	E6
A4	Added the pertinent negative for metformin tolerance, which supports continuing the drug unchanged.	**Completeness**	*Dr. Thorne: "you haven't reported any stomach upset."*	E6
A5	Replaced "requested travel clearance" with what the patient actually asked. No clearance was requested.	**Clinical specificity**	*Mrs. Vance: "Is there anything special I need to do with my meds for traveling?"*	E7

Objective (supplementary review)

Objective is outside the three required sections. It is included because two Assessment corrections depend on values recorded here.

- **Vital Signs:** BP: 118/74 mmHg (office).
- Home BP Log: average approximately 114/70 mmHg; 142/88 mmHg before the lisinopril increase. A6
- **Labs and Monitoring:** HbA1c: 6.8% (from the week before this visit) (Down from 7.4%). Fasting glucose log average: 115 mg/dL. A7

ID	Correction and reason	Standard	Transcript evidence	Log
A6	Added the home BP average and the pre-increase BP, and labeled 118/74 mmHg as the office reading. These values are the basis for the dose reduction.	**Completeness**	*"Your home readings have averaged around 114/70 mmHg. That's a significant drop from 142/88 mmHg before we increased the Lisinopril."*	E6
A7	Added HbA1c timing. Spoken values were verified. Other vital signs ("today's vitals") were never spoken aloud and must be entered from the vitals flowsheet, not inferred.	**Clinical specificity**	*"Your HbA1c from last week was 6.8%"; "Let's look at your home log and today's vitals."*	E10

Assessment

1. **Essential Hypertension:** ~~Controlled, but the patient is experiencing orthostatic symptoms secondary to recent antihypertensive titration. Blood pressure is currently lower than the target range.~~ Blood

pressure lower than needed on lisinopril 20 mg daily (office 118/74 mmHg; home average approximately 114/70 mmHg; 142/88 mmHg before the dose increase about 4 weeks ago). New orthostatic dizziness, attributed by the clinician to the lower blood pressure following the dose increase. A8

2. **Type 2 Diabetes Mellitus** ~~without complications~~ A9: ~~Well-controlled on current oral agent regimen.~~ Improved glycemic control on metformin ER 1000 mg twice daily: HbA1c 6.8% (from 7.4%); fasting glucose log average approximately 115 mg/dL; no GI upset reported. A10

ID	Correction and reason	Standard	Transcript evidence	Log
A8	Replaced the self-contradictory "Controlled, but… lower than the target range" with the drug, dose, BP values and the clinician's attribution. Kept the clinician's term "orthostatic dizziness." "Orthostatic hypotension" is not used because no orthostatic vital signs were recorded.	**Clinical specificity**	*"It's a bit lower than we need, which explains the lightheadedness"; "That should resolve the orthostatic dizziness."*	E6
A9	Struck "without complications." Complication status was never discussed, and this phrase drove the E11.9 code. Restore only if Dr. Thorne confirms.	**Clinical specificity**	*No discussion of diabetic complications anywhere in the transcript.*	E8
A10	Named the drug, dose and supporting data in place of "current oral agent regimen." Replaced "well-controlled" with the clinician's own assessment, backed by values.	**Clinical specificity**	*"HbA1c from last week was 6.8%, down from 7.4%. The Metformin ER 1000 mg twice daily is working well."*	E8

Plan

1. **Essential Hypertension:**
   - Decrease Lisinopril from 20 mg daily to ~~5 mg~~10 mg daily starting tomorrow morning (10/15/2026) to resolve orthostatic dizziness while keeping blood pressure in target range. A11
   - ~~Patient educated on rising slowly from seated/supine positions to prevent orthostatic lightheadedness.~~ A12
   - Patient repeated back the new dose: "decrease Lisinopril back to 10 mg daily." A13
2. **Type 2 Diabetes Mellitus:**
   - Continue Metformin ER 1000 mg twice daily.

3. Travel (2-week Alaska cruise next month):
   - Advised to pack medications in carry-on bag, bring a few extra days' supply and keep glucose testing supplies accessible. Clinician advised patient is fine to travel. A14

~~3~~4. Follow-Up and Disposition:

- Patient to return to clinic in ~~6 months for routine chronic care follow-up~~8 weeks to recheck blood pressure on lisinopril 10 mg daily and review basic metabolic panel. A15
- Instructed to call the clinic right away if dizziness ~~worsens or fails to resolve~~does not improve after lowering the lisinopril dose. A16

ID	Correction and reason	Standard	Transcript evidence	Log
A11	Corrected the new dose from 5 mg to 10 mg. Added the start date and the clinician's stated reason. Critical: this is a medication-dose error.	**Clinical specificity**	*"I'd like to step your Lisinopril back down from 20 mg daily to 10 mg daily starting tomorrow morning. That should resolve the orthostatic dizziness while keeping your blood pressure in a good target range."*	E1
A12	Deleted counseling that does not appear in the transcript. If Dr. Thorne confirms it was given but not captured, restore it with clinician attestation and reclassify E5 as capture loss.	**Clinical encounter relevance**	*The transcript explains why the dizziness happens but contains no instruction to rise slowly.*	E5
A13	Added the patient's read-back of the new dose, which shows the patient understood it.	**Completeness**	*Mrs. Vance: "Okay, decrease Lisinopril back to 10 mg daily. Got it."*	E1
A14	Added the travel counseling given in the visit. The note says "fine to travel" rather than "medically cleared" because the transcript reads "Mechanically" (see E9); the chosen wording holds under either reading.	**Completeness**	*"pack your medications in your carry-on bag, bring a few extra days' supply, and keep your testing supplies handy. Mechanically, you're fine to travel."*	E7, E9
A15	Corrected the interval from 6 months to 8 weeks and restored its purpose. "Six" came from the patient's question, not the clinician's answer. Critical: wrong interval delays monitoring of the dose change.	**Clinical specificity**	*Mrs. Vance: "three months or six?" Dr. Thorne: "Let's schedule a follow-up in 8 weeks to recheck your blood pressure on the 10 mg dose of Lisinopril and review a basic metabolic panel."*	E3
A16	Restored "right away" and the clinician's trigger (no improvement after the dose reduction).	**Clinical specificity**	*"If the dizziness doesn't improve after lowering the Lisinopril, call the clinic right away."*	E4

Verified without correction

The following draft content matches the transcript: patient identifiers and age (68 on 10/14/2026); office BP 118/74 mmHg; HbA1c 6.8% down from 7.4%; fasting glucose log average 115 mg/dL; metformin ER 1000 mg

twice daily continued unchanged; current lisinopril dose of 20 mg and the “tomorrow morning” start; the 2-week Alaska cruise next month; and the basic metabolic panel at follow-up.

Standards applied: Clinical specificity (9 corrections), completeness (4) and clinical encounter relevance (3).

Section 2 — Discrepancy Log

Ten discrepancies were found across the source transcript, the Dragon Copilot draft and the EMR chart. Each is assigned to the point in the documentation chain where it entered:

- **Capture loss:** the transcript did not capture what was said.
- **Draft error:** Dragon Copilot misrepresented, omitted or added to what the transcript records.
- **Edit error:** manual editing or transfer after the draft introduced a new problem.

Severity scale. High: could directly cause a wrong medication, order or appointment. **Moderate**: misrepresents the encounter or creates a coding or compliance risk. **Low**: a gap in source quality with no direct care impact once handled.

Summary

#	Error type	Specific content	Severity	Section 1 ref.
1	Draft error	Lisinopril new dose: 5 mg drafted; 10 mg ordered	High	A11, A13
2	Edit error	Medication Change/Orders field blank; 20 mg still active in EMR	High	Section 4
3	Draft error	Follow-up: 6 months drafted; 8 weeks ordered	High	A15
4	Draft error	Callback instruction lost “right away”	Moderate	A16
5	Draft error	Patient education not supported by the transcript	Moderate	A12
6	Draft error	Dose-increase context, symptom detail and BP trend missing	Moderate	A1–A3, A6, A8
7	Draft error	“Travel clearance” overstated; travel counseling omitted	Moderate	A5, A14
8	Draft error	“Without complications” unsupported; regimen not named	Moderate	A9, A10
9	Capture loss	“Mechanically”: probable mis-transcription of “Medically”	Low	A14
10	Capture loss	Vitals, log details and dates reviewed but not spoken	Low	A7

Pattern: both high-severity draft errors are numbers. The 5 mg dose appears nowhere in the transcript, and the 6-month interval came from the patient’s question rather than the clinician’s answer. Numeric values in AI drafts should be verified line by line against the source.

Entries

Entry 1 · Lisinopril new dose HIGH SEVERITY

Chain Location (Error Type): Draft error

Specific Content Affected: New lisinopril dose. Draft Plan 1: “Decrease Lisinopril from 20 mg daily to 5 mg daily.” Transcript, Dr. Thorne: “step your Lisinopril back down from 20 mg daily to 10 mg daily starting tomorrow morning”; patient read-back: “decrease Lisinopril back to 10 mg daily.” The value “5 mg” appears nowhere in the transcript.

Clinical or Record Impact: The signed record would show a 75% dose cut instead of the 50% cut ordered. A prescription or refill built from the note would under-treat hypertension that measured 142/88 mmHg before the last increase, and the record would contradict the dose the patient agreed to take. The error has already been pasted into the EMR Clinical Notes field (Finding 3.1).

Cross-reference: A11, A13

Entry 2 · Medication change not entered in discrete EMR fields HIGH SEVERITY

Chain Location (Error Type): Edit error

Specific Content Affected: Lisinopril order status in the EMR. Medication Change/Orders field: “[BLANK]: Field skipped during transfer.” Active Medication List: “Lisinopril 20 MG Oral Tablet (Active),” source “EMR Active Med Table (Unchanged).” The omission occurred during transfer into the EMR (a hand-pasted step, per the Clinical Notes field), after the draft was generated.

Clinical or Record Impact: The structured record says the patient is still on 20 mg, the dose linked to the new dizziness. Refills, pharmacy fills, interaction checks and the next medication reconciliation would all start from 20 mg. For a 68-year-old with orthostatic symptoms leaving on a two-week cruise, continued 20 mg dosing carries a fall risk. The chart now holds three different doses: 20 mg, 5 mg and 10 mg.

Cross-reference: Not correctable in the note; Section 4, Action 1

Entry 3 · Follow-up interval and purpose HIGH SEVERITY

Chain Location (Error Type): Draft error

Specific Content Affected: Return interval. Draft: “return to clinic in 6 months for routine chronic care follow-up and basic metabolic panel.” Transcript: “Let’s schedule a follow-up in 8 weeks to recheck your blood pressure on the 10 mg dose of Lisinopril and review a basic metabolic panel.” “Six” occurs only in the patient’s question (“three months or six?”), which the clinician did not adopt.

Clinical or Record Impact: A 6-month interval delays the BP recheck on the new dose and the metabolic panel by about four months; the panel monitors kidney function and potassium after an ACE-inhibitor dose change. The error has carried into the Structured Follow-Up Interval field and the AVS (Finding 3.5), so scheduling would book the wrong visit.

Cross-reference: A15

Entry 4 · Callback (safety-net) instruction MODERATE SEVERITY

Chain Location (Error Type): Draft error

Specific Content Affected: Draft: “call the clinic if dizziness worsens or fails to resolve.” Transcript: “If the dizziness doesn’t improve after lowering the Lisinopril, call the clinic right away.” The draft drops “right away” and the link to the dose reduction.

Clinical or Record Impact: The record shows less urgency than the patient was given. If the patient falls or delays calling, the chart would not show that prompt contact was advised. The instruction is also missing from the AVS (Finding 3.6).

Cross-reference: A16

Entry 5 · Unsupported patient education MODERATE SEVERITY

Chain Location (Error Type): Draft error

Specific Content Affected: Draft Plan 1: “Patient educated on rising slowly from seated/supine positions to prevent orthostatic lightheadedness.” The transcript explains the cause (“Dizziness upon standing can happen when blood pressure drops slightly with position changes”) but contains no instruction to rise slowly.

Clinical or Record Impact: The signed note would attest to counseling the source does not support. That is a compliance exposure. It is also a safety gap if the advice was never given, because the chart would show it as done. If Dr. Thorne confirms it was said but not captured, reclassify as capture loss and restore with clinician attestation.

Cross-reference: A12

Entry 6 · Medication-change context and symptom detail missing MODERATE SEVERITY

Chain Location (Error Type): Draft error

Specific Content Affected: (a) The lisinopril increase from 10 mg to 20 mg about 4 weeks ago, the stated reason for the visit, is absent, and the HPI calls the visit “routine.” (b) Dizziness: the transcript says “new,” “when I stand up quickly” and “passes after a few seconds”; the draft says “mild,” a word the patient never used. (c) The BP trend (142/88 before the increase; office 118/74; home average approximately 114/70 mmHg) is missing. (d) Assessment 1 calls BP both “Controlled” and “lower than the target range.”

Clinical or Record Impact: The note does not support the decision to cut the dose. A covering clinician, coder or auditor cannot see that the dizziness followed a dose increase or that BP fell below the clinician’s intended range. “Routine” and “mild” understate the medical decision-making, which weakens support for the visit’s billing level.

Cross-reference: A1–A3, A6, A8

Entry 7 · Travel discussion misstated; counseling omitted MODERATE SEVERITY

Chain Location (Error Type): Draft error

Specific Content Affected: Draft HPI: “requested travel clearance and medication management advice.” Transcript: “Is there anything special I need to do with my meds for traveling?” No clearance was requested. Dr. Thorne’s three instructions (pack medications in carry-on, bring a few extra days’ supply, keep testing supplies handy) appear nowhere in the draft Plan.

Clinical or Record Impact: “Travel clearance” implies a formal fitness-to-travel decision that a cruise line or insurer might rely on. The counseling actually given is undocumented in the note but partly appears in the

AVS (Finding 3.3), so the signed note and the patient's written instructions disagree. The cruise falls inside the 8-week recheck window, so the medication-supply advice bears directly on the new dose.

Cross-reference: A5, A14

Entry 8 · Diabetes complication status and regimen MODERATE SEVERITY

Chain Location (Error Type): Draft error

Specific Content Affected: Draft Assessment 2: "Type 2 Diabetes Mellitus without complications… Well-controlled on current oral agent regimen." The transcript never addresses diabetic complications, and it names the regimen (metformin ER 1000 mg twice daily) that the draft leaves generic. The EMR's E11.9 code was auto-populated from this wording (Finding 3.2).

Clinical or Record Impact: An unverified complication status becomes coded data. If the patient has a documented complication, E11.9 under-codes it and distorts risk adjustment and quality reporting. "Current oral agent regimen" sends the next reader elsewhere to find the drug and dose.

Cross-reference: A9, A10

Entry 9 · Probable transcription error: "Mechanically" LOW SEVERITY

Chain Location (Error Type): Capture loss

Specific Content Affected: Transcript: "Mechanically, you're fine to travel." In context this is most likely a speech-recognition substitution for "Medically." The transcript has been flagged, not altered; Dr. Thorne should confirm the wording.

Clinical or Record Impact: The retained source contains a clinically meaningless statement on the same topic the draft overstated as "travel clearance" (Entry 7). An auditor using the transcript as evidence could not confirm what was said. Section 1 uses "fine to travel," which is supported under either reading.

Cross-reference: A14

Entry 10 · Encounter data reviewed but not spoken aloud LOW SEVERITY

Chain Location (Error Type): Capture loss

Specific Content Affected: Dr. Thorne said "Let's look at your home log and today's vitals," but only the office BP and the log averages were spoken. Not captured: heart rate and weight; the number and date range of home readings; the HbA1c collection date ("last week"); and the date of the lisinopril increase ("about four weeks ago").

Clinical or Record Impact: The draft's Objective section is incomplete because its source is incomplete. Heart rate matters when assessing dizziness on standing, and weight is a standard diabetes measure. These values must come from the vitals flowsheet and lab results rather than be inferred, and the AI note should not be treated as a complete record of objective data.

Cross-reference: A7

Section 3 — EMR Record Integrity Evaluation

Field Appropriateness

Finding 3.1 · Medication change in the wrong field

Observation: The lisinopril change appears only as free text in the Clinical Notes field, reading “20 mg daily to 5 mg daily.” The Medication Change/Orders field is blank (“Field skipped during transfer”); the Active Medication List still shows “Lisinopril 20 MG Oral Tablet (Active).”

Implication: Fields that drive prescribing, refills and reconciliation say the patient remains on 20 mg; the only record of the change is narrative, at the wrong dose (Entries 1–2).

Finding 3.2 · Codes not clinician-selected

Observation: Both diagnosis codes are “AI Draft Auto-Populated.” E11.9 asserts “without complications,” which the encounter never addressed (Entry 8). No code reflects the orthostatic dizziness that drove the medication change.

Implication: Coded data states an unverified complication status and omits the reason the medication changed.

Traceability

Finding 3.3 · AVS content not traceable

Observation: The AVS reads “Pack medications in carry-on for Alaska cruise trip,” citing “AI HPI and Plan” as its source. Neither section contains that instruction.

Implication: Patient-facing text cannot be traced to the note being signed, and the signed note omits counseling the patient received in writing (Entry 7).

Finding 3.4 · No verification trail

Observation: Five of eight fields have an AI-draft origin; none records clinician verification. The pasted Clinical Notes text skips from Plan item 1 to item 3 with an ellipsis, so transfer of the metformin and callback items cannot be confirmed.

Implication: An auditor cannot separate verified from unverified content; the wrong values (5 mg, 6 months) carry the same provenance as correct ones.

Downstream Usability

Finding 3.5 · Follow-up field books the wrong visit

Observation: The Structured Follow-Up Interval field and the AVS both state 6 months; the clinician specified 8 weeks (Entry 3). No structured field holds the metabolic panel.

Implication: Scheduling would book about four months late, delaying the BP recheck and metabolic panel after an ACE-inhibitor dose change.

Finding 3.6 · Current dose cannot be determined

Observation: The AVS has no medication or callback instruction. A pharmacist or covering clinician would find three lisinopril doses: 20 mg (medication list), 5 mg (note) and 10 mg (transcript only).

Implication: The current dose cannot be determined from the EMR, and the patient’s written instructions omit both the dose change and the safety-net instruction.

Section 4 — Disposition Recommendation

Recommendation: ESCALATE

Same-day clinician review; not for the routine sign-off queue.

Rationale: Editing the note alone would leave the record unsafe. Entry 1 (5 mg drafted, not 10 mg) and Entry 2 (blank Medication Change/Orders field; 20 mg still active), confirmed by Findings 3.1 and 3.6, create a three-way dose conflict that only clinician order entry can resolve. Entry 3 and Finding 3.5 show the wrong interval has already reached scheduling and the AVS. Findings 3.3–3.4 show AI content reached patient-facing and coded fields unverified.

Required actions before sign-off:

1. Dr. Thorne enters or confirms lisinopril 10 mg daily and discontinues 20 mg (Entries 1–2; Finding 3.1).
2. Correct follow-up to 8 weeks with BP recheck and metabolic panel; update the structured field; reissue the AVS with the new dose and “call right away” instruction (Entries 3–4; Findings 3.5–3.6).
3. Dr. Thorne confirms Entries 5, 8 and 9 (counseling, complication status, “Mechanically”).
4. Coordinator applies Section 1 corrections and re-verifies against the transcript before sign-off.
5. Refer Findings 3.3–3.4 to the quality lead before the compliance review.

Review Attestation

Reviewed against the source transcript, Dragon Copilot draft and EMR post-transfer chart for ENC-2026-1014-884.

Reviewer (Clinical Office Coordinator) Date

Clinician acknowledgment (Marcus Thorne, MD) Date